This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Saxenda (Liraglutide 6mg/ml) solution for injection in pre-filled pen for the treatment of Weight management in adults

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Weight management in adults

NICE Guidance on Obesity and Weight Management

Weight management interventions are recommended for adults with a body mass index (BMI) ≥30 kg/m², or ≥27 kg/m² with weight-related comorbidities. Lifestyle modifications including reduced calorie intake and increased physical activity are the cornerstone of weight management. Pharmacological intervention with anti-obesity drugs, such as GLP-1 receptor agonists (e.g., liraglutide), may be considered as an adjunct to lifestyle changes in eligible patients.

Liraglutide (Saxenda) Overview

- GLP-1 (Glucagon-Like Peptide-1) receptor agonist indicated for chronic weight management
- Mechanism of action: GLP-1 receptor agonist; promotes satiety, delays gastric emptying and improves glycaemic control
- Administered as subcutaneous injection once daily

Indication and Patient Selection

- **Eligibility criteria:** Adults aged 18 years and over with initial BMI ≥30 kg/m² or ≥27 kg/m² with at least one weight-related comorbidity such as dysglycaemia, hypertension, dyslipidaemia, or obstructive sleep apnoea
- Patients must be motivated to achieve weight loss and committed to lifestyle modifications (reduced-calorie diet and increased physical activity)

Efficacy and Expected Outcomes

- Clinical trials demonstrate approximately 5-10% body weight reduction from baseline in responders
- Response is assessed at 12 weeks on the maintenance dose (3.0 mg daily); discontinue if <5% body weight loss achieved
- Improvements in weight-related comorbidities (glycaemic control, blood pressure, lipid profile) may be observed

Important Safety Considerations

- Absolute contraindication: Personal or family history of medullary thyroid carcinoma (MTC) or Multiple Endocrine Neoplasia syndrome type 2 (MEN 2)
- Caution in patients with a history of pancreatitis or with mild to moderate renal or hepatic impairment. Under this PGD inflammatory bowel disease, diabetic gastroparesis, severe renal impairment (eGFR below 30 mL/min/1.73m2) and severe hepatic impairment (Child-Pugh C) are exclusions
- Risk of hypoglycaemia increases when used with insulin or sulfonylureas; dose adjustment of these agents may be required
- Common adverse effects include gastrointestinal symptoms (nausea, vomiting, diarrhoea, constipation), which typically diminish over time

Patient Group Direction

for the supply/administration of

Saxenda (Liraglutide 6mg/ml) solution for injection in pre-filled pen

for the treatment of

Weight management in adults

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines

	• All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Adjunct to reduced-calorie diet and increased physical activity for weight management in adults with initial BMI ≥30 kg/m² or ≥27 kg/m² with at least one weight-related comorbidity (e.g., dysglycaemia, hypertension, dyslipidaemia, obstructive sleep apnoea)
Inclusion criteria	Age 18 years and over BMI ≥30 kg/m² or ≥27 kg/m² with at least one weight-related comorbidity Willing and motivated to undertake lifestyle modifications (reduced-calorie diet and increased physical activity) Informed consent obtained No contraindications as listed in exclusion criteria
Exclusion criteria	Known hypersensitivity to liraglutide or any component of the product

	Personal or family history of medullary thyroid carcinoma (MTC) Personal or family history of Multiple Endocrine Neoplasia syndrome type 2 (MEN 2) Pregnancy or breastfeeding Severe heart failure (NYHA class IV) Severe hepatic impairment (Child-Pugh score C or equivalent) Severe renal impairment (eGFR <30 mL/min/1.73m^2) Concurrent use of other GLP-1 receptor agonists Any other weight-management medicine, current: tirzepatide, semaglutide, orlistat or naltrexone/bupropion Current or previous eating disorder Obesity secondary to an endocrine disorder, or to a medicine that causes weight gain Aged 75 years or over (use not recommended, SmPC) Type 1 diabetes, or insulin-treated diabetes. Refer. Any history of pancreatitis while taking a GLP-1 receptor agonist Inflammatory bowel disease, or diabetic gastroparesis (use not recommended, SmPC) Recent (within 3 months) acute pancreatitis or chronic pancreatitis with ongoing risk factors Acute illness or recent surgery
Cautions	History of pancreatitis not caught by the exclusions above (more than 3 months ago, not while taking a GLP-1 receptor agonist, and no ongoing risk factors): risk of recurrence; counsel and consider referral Mild to moderate renal impairment (eGFR 30-60 mL/min/1.73m^2) Mild to moderate hepatic impairment Other gastrointestinal disorders (diabetic gastroparesis and inflammatory bowel disease exclude) Thyroid disease (monitor thyroid function if symptoms develop) Sulfonylureas - hypoglycaemia risk; inform the prescriber (insulin-treated diabetes excludes) History of depression or suicidal ideation (monitor mental health) Significant dehydration (acute illness is an exclusion, see exclusion criteria) Concurrent use of medications affecting gastric motility
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed.

	Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Liraglutide 6mg/ml solution for injection in pre-filled pen
Legal category	POM (Prescription Only Medicine)
Storage	Before first use: Store at 2-8°C in a refrigerator. After first use: Store below 30°C for up to 30 days. Do not freeze. Protect from light. Do not use if the solution is discoloured or contains particles.
Route/method of administration	Subcutaneous injection into the abdomen, thigh, or upper arm. Inject at any time of day, preferably the same time each day. Not to be administered intravenously (IV) or intramuscularly (IM).
Dose and frequency	Titration schedule (weekly dose steps; the maintenance dose is reached in week 5): • Week 1: 0.6 mg once daily (subcutaneous injection) • Week 2: 1.2 mg once daily • Week 3: 1.8 mg once daily • Week 4: 2.4 mg once daily • Week 5 onwards: 3.0 mg once daily (maintenance dose) If dose escalation is not tolerated, consider delaying by approximately one week before continuing titration. Maintenance dose: 3.0 mg once daily by subcutaneous injection
Quantity to be administered and/or supplied	Up to 5 pre-filled pens per supply (approximately 1 month at maintenance dose; each pre-filled pen contains 18 mg liraglutide)
Maximum or minimum treatment period	Review at 12 weeks on the maintenance dose (3.0 mg daily). Discontinue if <5% body weight loss has been achieved. If treatment is continued, review at least every 6 months thereafter. Longer treatment duration may be considered in patients achieving sustained weight loss of ≥5% body weight.
Adverse effects	Very common (≥1/10): Nausea, vomiting, diarrhoea, constipation Common (≥1/100 to <1/10): Hypoglycaemia (primarily in patients with type 2 diabetes mellitus receiving concurrent glucose-lowering therapy), headache, dizziness, abdominal pain, dyspepsia, flatulence, injection site reactions (erythema, induration), fatigue, increased serum lipase Uncommon (≥1/1,000 to <1/100): Dehydration, tachycardia, cholelithiasis, cholecystitis, acute cholecystitis

	Serious (reportable): Pancreatitis (persistent severe abdominal pain with or without vomiting), anaphylaxis, angioedema, thyroid nodules or neoplasms, depression, suicidal ideation

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply, dose, form, route and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with Saxenda. Ensure the patient understands the injection technique, storage requirements, and when to seek medical advice.
Follow-up advice to be given to patient or carer	Inject using the correct subcutaneous technique. Rotate injection sites to avoid lipohypertrophy. Dispose of used needles safely in a sharps container. Take the injection at the same time each day for consistency. It can be taken at any time of day but choosing a fixed time may aid compliance. Continue with a reduced-calorie diet and increase physical activity as advised. Weight management is most effective when medications are combined with lifestyle modifications. Expect initial gastrointestinal side effects (nausea, vomiting) which typically improve over the first few weeks. Take the medication with food if nausea is problematic. Stay well-hydrated. Report persistent or severe abdominal pain immediately, as this may

	indicate pancreatitis. If you experience signs of hypoglycaemia (shakiness, sweating, confusion, rapid heartbeat) - particularly if taking insulin or sulfonylureas - inform your GP or healthcare provider, as dose adjustment of these medications may be needed. Inform your healthcare provider if you develop symptoms of thyroiditis (neck pain, swelling, difficulty swallowing) or depression/mood changes. Do not freeze the medication. Before first use, store at 2-8°C. After opening, store below 30°C for up to 30 days. Attend all review appointments. Weight loss will be monitored at 12 weeks, and treatment will be discontinued if <5% body weight loss is achieved. Do not share your medication with others. Saxenda is prescribed specifically for you based on your medical history and BMI.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Obesity: identification and management
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Cover title corrected from administration to supply; the service is a supply for self-injection.

Guidance summary: the mechanism line no longer attributes glycaemic control to the central nervous system.

Guidance summary: the caution bullet now states that inflammatory bowel disease, gastroparesis, severe renal and severe hepatic impairment are exclusions under this PGD, as the v002 change made them.

Cautions: the history-of-pancreatitis caution now states which pancreatitis histories it covers, so the boundary with the two pancreatitis exclusions is stated once.

Cautions: acute illness removed from the dehydration caution because acute illness is an exclusion.

Dose row: titration wording corrected (weekly steps, maintenance dose reached in week 5).

Records row: the two run-together bullets were separated and punctuated (date of supply, dose, form, route and quantity supplied; adverse reactions and actions taken; supplied via PGD).

Records row: the under-18 record-retention bullet was removed because this PGD does not treat anyone under 18.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

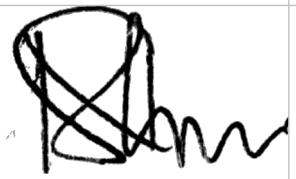
Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Exclusion criteria: the Saxenda SmPC states use is not recommended at 75 years and over, in inflammatory bowel disease and diabetic gastroparesis, with other weight-management medicines, in secondary obesity, and that liraglutide must not be restarted after pancreatitis; type 1 and insulin-treated diabetes cannot be managed in a pharmacy service. The previous version had these as cautions or did not mention them. All now exclusions.

GET REAL HEALTH

Signed on behalf of Get Real Health

Version v006, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.